

February 23, 2026

Department of Health and Human Services

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology (ASTP/ONC)

Attention: Request for Information – HHS Health Sector AI RFI

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Washington, DC 20201

Submitted electronically via upload to <http://www.regulations.gov>

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care, (RFI 2025-23641)

Dear Assistant Secretary Keane:

IQVIA appreciates the opportunity to respond to the ASTP/ONC Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.¹ As a global provider of advanced analytics, technology solutions, and clinical research services, we support healthcare organizations, regulators, and life-science companies across more than 100 countries. IQVIA Healthcare-grade AI® spans AI-enabled clinical development and decision support, safety surveillance, real-world evidence generation, and health-system modernization.

IQVIA has long been a leader in trustworthy and responsible AI as well as protecting patient privacy, from developing healthcare-grade AI evaluation frameworks. Key leadership innovations in AI includes:

- Creation of the *IQVIA LLM Trustworthiness Toolkit for Healthcare-grade AI*²
- Pioneering award-winning, privacy-preserving technologies, used by the UK's NHS and recognized by Bio-IT World in 2025³
- Development of the *IQVIA Synthetic Trends Engine*, an advanced analytics capability recognized with a PM360 award⁴ for enabling robust modeling and insight generation while supporting strong governance, transparency, and responsible AI practices.⁵

These innovations underscore IQVIA's commitment to advancing safe, transparent, and clinically reliable systems that can be confidently deployed in real-world care settings.

We have three policy recommendations based on our decades of experience in AI/ML-informed healthcare that we believe would reduce barriers to innovation, improve deployment of AI, and build trust in the use of AI among clinical organizations, clinicians, and their patients. We firmly believe that if HHS were to establish a single, risk-based lifecycle governance framework for AI and embed it across HHS regulations, standards, and reimbursement systems, healthcare stakeholders could adopt a harmonized, risk-based approach

¹ Federal Register. (2025, December 23). Request for information: Accelerating the adoption and use of artificial intelligence as part of clinical care (Document No. 2025-23641). <https://www.federalregister.gov/documents/2025/12/23/2025-23641/request-for-information-accelerating-the-adoption-and-use-of-artificial-intelligence-as-part-of>

² Nanavati, J., de Oliveira, R., Yacoubian, C., & Zine El Abidine, K. (2024, August 16). Making GenAI reliable: IQVIA LLM trustworthiness toolkit for healthcare-grade AI™. IQVIA. <https://www.iqvia.com/blogs/2024/08/making-genai-reliable>

³ Bio-IT World. (2025, March 19). Bio-IT World names innovative practices winners. NHS Clinical Research Network Project. <https://www.bio-itworld.com/news/2025/03/19/bio-it-world-names-innovative-practices-winners>

⁴ PM360. (2025). PM360 2025 innovative marketing technology or services: IQVIA Digital consumer modeled audiences. <https://pm360online.com/pm360-2025-innovative-marketing-technology-or-services-iqvia-digital-consumer-modeled-audiences/>

⁵ IQVIA. (2026, January 7). Trustworthy AI/ML for patient analytics and research. <https://www.iqvia.com/library/white-papers/trustworthy-ai-ml-for-patient-analytics-and-research>

spanning pre-deployment evaluation, deployment, and post-deployment monitoring of healthcare AI. This framework would reduce fragmentation by providing a clear national reference point and could incentivize states to voluntarily align their AI policies with HHS guidance, rather than developing overlapping or conflicting requirements that impede innovation at scale.

Overarching Policy Recommendations:

- **Continued commitment to consensus-based standards** for information sharing (interoperability), data security, and privacy regulations, reimbursement policy, and contracts. IQVIA commends ASTP/ONC for its long-standing commitment to the development and adoption of consensus-based standards and strongly encourages continuing this approach across HHS. Whenever feasible, using consensus-based standards will help to eliminate confusion and streamline AI adoption. For example, ASTP/ONC's proposed HTI-5 rule takes a FHIR-forward approach to facilitate data interoperability and streamline AI adoption.
- **Incorporate best practices for governance**, including AI life-cycle management, into regulations, guidance, and contracts. Life-cycle management of AI embeds oversight of risk and performance. We strongly urge HHS to provide a one-HHS policy document that outlines a staged life-cycle management framework for AI and then embed the framework, or parts of the framework as appropriate, in reimbursement policy, CMS provider certification requirements, ASTP/ONC's EHR certification program, and FDA guidance.
- **Incentivize use of AI in healthcare delivery, administration, and operations.** Reimbursement as well as certification standards, guidance documents and contracts should encourage use of AI with life-cycle management in delivery of care and the conduct of research.

IQVIA aligned its recommendations in response to the RFI with HHS leadership priorities to reduce regulatory burden, streamline oversight, and enable high-impact, low-risk use of AI in clinical care. Our approach emphasizes risk-based governance frameworks, reliance on consensus standards and the use of certification, interoperability, and payment incentives to allow market forces to shape innovation, while preserving patient trust, data integrity, and clinical accountability.

The sections below provide responses to some of the specific questions posed in the RFI from IQVIA experts and build on our overarching policy recommendations with targeted input aligned to HHS AI Strategy⁶ and ASTP/ONC priorities.^{7,8,9}

⁶ U.S. Department of Health and Human Services. (2025, December 3). HHS Artificial Intelligence Strategy & Implementation. <https://www.hhs.gov/programs/topic-sites/ai/strategy-implementation/index.html>

⁷ U.S. Department of Health and Human Services, Assistant Secretary for Technology Policy & Office of the National Coordinator for Health Information Technology. (2024). 2024–2030 Federal health IT strategic plan.

⁸ U.S. Department of Health and Human Services, Office of the National Coordinator for Health Information Technology. (2024). Health data, technology, and interoperability: Certification program updates, algorithm transparency, and information sharing (Final rule). Federal Register, 89(6), 1192–1354. <https://www.federalregister.gov/documents/2024/01/09/2023-28857/health-data-technology-and-interoperability-certification-program-updates-algorithm-transparency-and>

⁹ Department of Health and Human Services, Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology. (2025, December 29). Health data, technology, and interoperability: ASTP/ONC deregulatory actions to unleash prosperity (Proposed rule, 90 Fed. Reg. 60970). <https://www.federalregister.gov/documents/2025/12/29/2025-23896/health-data-technology-and-interoperability-astponc-deregulatory-actions-to-unleash-prosperity>

Question 1: Barriers to Innovation

What are biggest barriers to private sector innovation in AI adoption and use in clinical care?

At IQVIA, we have seen that private-sector innovation in clinical-care AI is constrained by a combination of structural, regulatory, economic, and trust-related barriers that limit the development, validation, and scalable deployment of AI tools across health-care settings. We commend ASTP/ONC in the release of the HTI rule series and for establishing important transparency and risk-management expectations within certified health IT. We encourage ASTP/ONC to continue coordinating towards a cross-HHS vision for AI lifecycle management that spans regulation, reimbursement, and research. Without such a vision, and incorporation of the life-cycle management framework, as appropriate, into HHS policies, we would expect fragmentation and continued barriers to AI development and use in healthcare.

Collectively, these barriers cluster into four areas: (1) infrastructure and data constraints, including EHR integration burden, vendor-specific deployment pathways, limited interoperability, fragmented and locally inconsistent data, and limited access to diverse real-world care settings for validation; (2) regulatory and governance fragmentation, including pathways and oversight approaches not well-suited to adaptive or generative AI, inconsistent security and privacy requirements across institutions, and continued uncertainty about accountability when AI contributes to clinical decision-making; (3) market and economic uncertainty, particularly inconsistent reimbursement and unclear return on investment that keeps many AI tools in pilot mode and limits sustained investment in product development and operational integration; and (4) evidence and trust gaps, driven by inconsistent expectations for monitoring, documentation, version control, auditability, and real-world performance evidence, coupled with limited infrastructure to generate these artifacts at scale. These challenges reinforce one another and are exacerbated when AI lifecycle expectations are not harmonized across regulation, reimbursement, and research. Addressing these barriers requires clear, harmonized lifecycle management expectations and reduced friction across existing HHS authorities.

Question 2. Regulatory, Payment, and Programmatic Design Changes HHS to Prioritize

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

IQVIA has been working to advance the effective use of AI in clinical care by modernizing governance frameworks, setting clear operational expectations, and creating strong economic incentives for our customers. Policies built for static software do not reflect the behavior, risk profile, or evidence needs of adaptive, generative, and agentic AI systems and should not be used for healthcare AI. Rather, instituting best practice governance and lifecycle management increases predictability for developers and healthcare organizations while bolstering patient safety. We recommend future ASTP/ONC and HHS policies establish transparent, standards-based expectations and enable coordination across HHS, in lieu of prescriptive, model-specific regulatory requirements. ASTP/ONC's role would be to convene stakeholders, signal federal expectations, and promote alignment across agencies under existing authorities related to certification, interoperability, transparency, and programmatic incentives to streamline oversight while enabling safe innovation at scale. For example:

- **Modernize and Harmonize Regulatory Oversight for Dynamic AI.** Many current oversight pathways were built for static software and do not align with adaptive, generative, or agentic AI systems. FDA's "special controls" and pre-market evidence expectations, such as multi-reader, multi-case (MRMC) studies, introduce burdens that slow iteration and limit developers' ability to evaluate AI in its intended context. A shift toward evidence-based, lifecycle-oriented oversight, balancing pre-market expectations with structured post-deployment monitoring and real-world evidence, would better reflect how AI interacts with evolving clinical workflows and data. This

approach emphasizes tailoring controls to demonstrated risk and performance, rather than default reliance on rigid or static special controls that may unintentionally impede innovation.

In addition, clearer expectations for evaluating generative and agentic behaviors are needed to support safe deployment. Aligning oversight with established frameworks, such as the NIST RMF, ISO/IEC 42001, and CTA-2135, would reduce fragmentation and create more predictable, standards-based expectations across FDA and HHS more broadly. ASTP/ONC is well positioned within its existing certification, interoperability, and transparency authorities to convene stakeholders, signal shared expectations, and promote alignment across FDA, CMS, and other HHS components helping ensure that expectations for transparency, monitoring, and change management are consistent across the AI lifecycle.

- **Prioritize Clinical Problem-Solving Over Regulatory Process.** While FDA clears many AI-enabled devices each year, clearance often does not translate into adoption or reimbursement by CMS. Market authorization alone is insufficient to ensure clinical uptake, particularly for AI functionality. Manufacturers typically design AI devices based on requirements gathered from clinicians, including physicians and nurses. However, FDA review processes frequently override these clinical requirements, effectively forcing manufacturers to choose between market clearance and clinician-driven design. This one-size-fits-all approach limits functionality and contributes to an innovation gap, as similar devices often achieve market entry in the EU and other markets without significant safety concerns. These challenges persist even for devices designated under the FDA Breakthrough Devices Program. In many cases, changes required during FDA review alter the device's intended use, ultimately disqualifying it from CMS breakthrough reimbursement pathways and undermining the purpose of the designation. HHS can play a role in aligning NIH, FDA, and CMS incentives and requirements. Improved oversight and streamlined interagency processes would help ensure that innovative AI devices that address real clinical needs are not stalled between regulatory clearance and reimbursement, and that clinically valuable technologies reach patients. ASTP/ONC-led coordination across HHS can help ensure that transparency, interoperability, and lifecycle governance expectations established at clearance do not become barriers to downstream reimbursement and clinical adoption.
- **Strengthen Governance Through Transparency, Privacy Clarity, and Safe-Harbor Approaches.** Clear governance expectations are essential for consistent and trustworthy AI deployment. For example:
 - Building on ASTP/ONC's existing transparency framework under 45 CFR Part 170, similar transparency expectations for non-device AI could be advanced through voluntary certification pathways, federal procurement standards, and other non-regulatory mechanisms to ensure clinicians and implementers have access to provenance, performance summaries, version history, and change documentation.
 - Clearer expectations around continuous risk assessment, including drift detection, monitoring intervals, update documentation, and performance reassessment as models operate in real-world conditions. For organizations that maintain robust AI management systems aligned with NIST or ISO standards, HHS could explore shared-responsibility safe-harbor models that reduce uncertainty and support more predictable compliance expectations when strong governance and auditability controls are demonstrably in place.

Such approaches align with ASTP/ONC's emphasis on voluntary, standards-based transparency and certification-driven trust.

- **Modernize Payment Policy to Incentivize Preventive, AI-Enabled Care.** Many high-value clinical AI tools support early identification of rising-risk patients or emerging care gaps, yet fee-for-service structures provide limited incentives for proactive preventative care. More predictable pathways for

reimbursing validated, AI-enabled preventive services, like those being piloted by CMMI in ACCESS and BALANCE, would help overcome this structural barrier and better align incentives with population-health goals. Emerging CPT codes provide initial clarity and payment via the Medicare Physician Fee Schedule (MPFS) should advance more rapidly to speed clinical uptake. Clear ASTP/ONC expectations around transparency, lifecycle governance, and interoperability can reduce uncertainty for CMS and commercial payers, enabling faster movement from pilots to reimbursed clinical use. Stable and transparent payment signals would encourage health systems to move AI tools beyond pilots and toward sustained, outcomes-focused deployment.

Question 4. Evaluation

What are the most promising AI evaluation methods (pre- and post-deployment) for clinical care? Should HHS further support these (evaluation) processes? If so, how?

IQVIA's operational models strengthen AI evaluation by expanding support for real-world testing, reduce fragmentation across partnership expectations (harmonization and use of consensus-based standards and frameworks), and lowering operational barriers for our healthcare organizations partners. These IQVIA operational models may align directly with ASTP/ONC's priorities to enable trustworthy AI adoption through lifecycle governance, interoperability, transparency, and cross-HHS coordination rather than model-level regulation. In this light, we suggest the following concepts to HHS:

Expand funding for real-world and workflow-embedded evaluation. Support for prospective pilots, EHR-integrated testing, and post-deployment monitoring infrastructure would help ensure evaluations reflect the complexity of real clinical environments and the constraints clinicians face. Reimbursement systems should provide incentive payments for entities working to embed AI and AI life-cycle management.

Standardize evaluation frameworks across HHS divisions. Common expectations for lifecycle documentation, real-world evidence, monitoring frequency, and transparency would improve consistency and allow developers and healthcare organizations to design more reliable evaluation strategies. Embed requirements for best practice life-cycle management and governance into standards updates and certification programs connected to CMS reimbursement policies; FDA market approval guidance; and ASTP/ONC EHR certification.

Incentivize transparent reporting. HHS can encourage programs that support clear reporting of methods, datasets, and performance results, including subgroup and drift analyses, so that buyers and clinicians can meaningfully compare solutions by providing incentive payments for the use of such tools.

Use Other Transaction Authority (OTA) to accelerate evaluation through public-private partnership. OTA can support collaboration with industry on shared test beds, demonstration projects, and iterative evaluation in real-world settings, enabling faster timelines, flexible IP arrangements, and adaptive evaluation designs that reduce participation risk while preserving rigor. OTA's flexibility can also de-risk participation by enabling alternative IP arrangements, faster timelines, and more adaptive evaluation designs than traditional procurement allows.

Together, these actions would help build a more consistent, clinically grounded evaluation ecosystem, one that enables high-quality AI tools to demonstrate safety and effectiveness while supporting responsible adoption across diverse care settings.

Question 5: Supporting Innovation & Use

How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing)?

HHS can strengthen private-sector accreditation, certification, and industry-driven testing by setting clearer expectations for evaluation, supporting shared testing infrastructure, and reducing barriers that impede the deployment of trustworthy AI tools. As noted earlier, use of consensus-based standards in all regulations that incent or regulate health care use of data and AI tools will begin to create a cohesive and clear path for governance and life cycle management.

Establish National, Risk-Stratified Validation Expectations. HHS can help coordinate private-sector assurance efforts by outlining risk-stratified validation expectations that specify the types of evidence AI developers should produce, by setting standards that must be met if AI is to be reimbursed or deployed for operations via CMS provider certification. This includes technical validation (accuracy, robustness, calibration, fairness, and known failure modes), clinically meaningful evaluation, and post-deployment monitoring requirements such as drift detection, change control, and update documentation.

Align Certification Pathways with Established Standards. HHS can support private certification bodies by endorsing a low-overhead certification pathway grounded in best practices for data validation, interoperability, workflow integration, testing, and bias monitoring. Aligning this pathway established standards would provide a common foundation for assurance activities and simplify how developers and HCOs demonstrate conformity to best practices. Setting use of consensus-based frameworks or standards at the bar for reimbursement and provider certification will bring rigor to the use of AI in healthcare.

Incentivize Adoption of Certified, Standards-Aligned Tools. HHS can increase the impact of private-sector certification by offering incentives for HCOs to adopt interoperable, standards-aligned AI tools. Targeted grants, reimbursement advantages, or preferential access to federal pilot programs would encourage organizations to adopt tools that meet established certification criteria.

Support Neutral, Independent Evaluation Environments. HHS can enhance private-sector evaluation capacity by supporting neutral, independent testing environments where auditors can conduct standardized assessments of robustness, bias, and documentation quality. Shared evaluation sandboxes and procurement-ready conformance checklists would reduce duplicative testing and strengthen the reliability of private accreditation and certification efforts.

Taken together, these actions would help harmonize evaluation practices, lower adoption barriers for health systems, and enable private-sector innovation to progress within a consistent, standards-aligned framework.

Question 7: HCO Decision makers

Which role(s), decision maker(s), or governing bodies within healthcare organizations (HCOs) have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Roles and Decisionmakers Influencing AI Adoption in Clinical Care. AI adoption in clinical care depends on a mix of clinical, operational, and governance authorities. Key decisionmakers typically include Chief Medical, Nursing, Information, and Informatics Officers; CIOs and CTOs; and leaders overseeing Quality, Patient Safety, Compliance, Privacy, and Security. Many health systems also maintain multidisciplinary AI or clinical-informatics review boards responsible for evaluating evidence, workflow fit, and ongoing monitoring. Innovation, informatics, and implementation teams further assess technical feasibility and determine whether tools can be configured within vendor-constrained EHR environments.

While governance groups shape formal approval pathways, clinician endorsement often determines whether AI becomes embedded in routine care. Sustained use depends on whether tools reduce cognitive burden, fit naturally within existing workflows, and provide value at the point of care.

Patients also influence organizational decisions, especially for AI systems supporting shared decision-making, providing personalized risk information, or using patient-generated data. In many organizations, patient advocates serve on advisory councils or governance committees, ensuring that patient priorities, such as usability, transparency, accessibility, and data stewardship, are represented in deliberations. As AI capabilities expand (e.g., personalized risk estimation, digital navigation support), patient-experience perspectives increasingly shape whether tools are deemed appropriate for deployment.

Primary Administrative Hurdles. Administrative barriers frequently stem from fragmented decision-making processes, limited technical capacity, and workflow pressures. IT and clinical-informatics teams are often stretched by core operational priorities, such as EHR upgrades, security requirements, and regulatory compliance, leaving limited bandwidth to support integration, monitoring, or iterative refinement of AI tools. Vendor-specific integration pathways, restrictions on customization, and the need to maintain certification further complicate deployment and increase the time and cost required to bring tools online.

Governance-related challenges also contribute to slow adoption. Many organizations lack clear procedures for AI-specific data approvals, lifecycle oversight, version control, and post-deployment monitoring. Risk-averse procurement processes and unclear accountability for ongoing model performance add to delays, even in cases where the clinical value is well demonstrated.

For tools involving patient-generated data or personalized insights, administrative teams must update consent workflows, strengthen transparency processes, and ensure adequate privacy safeguards. These adjustments require coordination across clinical, legal, IT, and patient-experience teams, adding another layer of complexity.

These decision-making dynamics and administrative hurdles align with ASTP/ONC's focus on transparency, interoperability, lifecycle governance, and human-centered integration as the primary levers to reduce friction for adoption and support consistent, scalable use of AI within healthcare settings.

Concluding Comments: Role of Public–Private Collaboration

Organizations like IQVIA can partner with HHS to operationalize AI for healthcare by supporting both the shared infrastructure and the methodological foundations required for responsible adoption. Through global data assets, analytics platforms, and experience managing multi-organization networks, IQVIA can help design and operate interoperable infrastructure that enables secure, scalable collaboration. Complementing this, IQVIA can provide methodological expertise to inform evaluation frameworks, develop practical semantic standards, and surface high-value research needs, helping translate public–private collaboration into measurable impact in clinical care.

IQVIA is already demonstrating the value of these collaborations through active partnerships with NVIDIA¹⁰ and AWS,¹¹ which focus on agentic AI development, reasoning-safe LLM architectures, and scalable, privacy-preserving compute. These partnerships directly address the research gaps identified above, shared infrastructure, evaluation frameworks, and secure-by-design AI, and offer a practical model for how federal investment and public-private collaboration can accelerate safe clinical deployment.

IQVIA has developed more than 200 AI algorithms across diverse clinical contexts, with a particular emphasis on identifying undiagnosed and misdiagnosed patients to accelerate disease detection and diagnostic innovation. More than 40 scientific articles and conference presentations document these

¹⁰ Shah, V. (2025, June 11). IQVIA and NVIDIA harmonize to accelerate clinical research and commercialization with AI agents. NVIDIA Blog. <https://blogs.nvidia.com/blog/iqvai-ai-agents-clinical-research/>

¹¹ IQVIA. (2025, December 2). IQVIA announces strategic collaboration with AWS, naming AWS as preferred agentic cloud provider to power next-generation AI platform. <https://www.iqvai.com/newsroom/2025/12/iqvai-announces-strategic-collaboration-with-aws>

approaches, including models for Type 1 diabetes,^{12,13} idiopathic pulmonary arterial hypertension,¹⁴ alpha-1 antitrypsin idiopathic pulmonary fibrosis,¹⁵ and non-alcoholic steatohepatitis¹⁶ across structured and unstructured data sources. These algorithms have undergone clinical, analytical, and commercial validation, and include interpretable machine-learning models using NLP, longitudinal patient trajectories, and multimodal real-world datasets such as EHRs and medical claim.^{17,18,19}

IQVIA's models are currently being used and trained by healthcare organizations, including NHS England and U.S. healthcare providers. Together, these examples demonstrate how to advance ASTP and ASTP/ONC priorities related to scalable evaluation, workforce augmentation, interoperability, and public trust—while delivering measurable clinical and operational value.

Thank you for the opportunity to contribute to this important effort. We view this RFI as a meaningful step toward a coordinated, trustworthy approach to AI in clinical care and appreciate ASTP and ASTP/ONC's leadership in advancing standards-based interoperability, transparency, and lifecycle governance. We would welcome continued engagement with ASTP/ONC as policies, standards, and certification programs evolve and are happy to provide additional details or connect you with IQVIA's subject-matter experts as helpful.

Respectfully submitted,

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¹² Brusini, I., Lee, S., Hollingsworth, J., Sees, A., Hackenberg, M., Scherpier, H., López-Díez, R., & Leavitt, N. (2025). Deploying machine learning models in clinical settings: A real-world feasibility analysis for a model identifying adult-onset type 1 diabetes initially classified as type 2. JAMIA Open, 8(5), ooaf133. <https://doi.org/10.1093/jamiaopen/ooaf133>

¹³ Breakthrough T1D. (2022, August 11). A new tool to accurately diagnose type 1 in adults. <https://www.breakthrough1d.org/news-and-updates/iqvia-algorithm-for-misdiagnoses/>

¹⁴ Kiely, D. G., Doyle, O., Drage, E., Jenner, H., Salvatelli, V., Daniels, F. A., et al. (2019). Utilising artificial intelligence to determine patients at risk of a rare disease: idiopathic pulmonary arterial hypertension. Pulmonary Circulation, 9, 2045894019890549. <https://doi.org/10.1177/2045894019890549>

¹⁵ Knight, A., Megrhi, S., Nigmatullina, Y., Glasser, J., Smyth, A., Keefer, J., Rigg, J. (2019). Use of Artificial Intelligence to Identify Undiagnosed Idiopathic Pulmonary Fibrosis Patients. In D12. ILD: Diagnosis and Prognosis. American Thoracic Society, A7453–A7453. <https://doi.org/10.1164/ajrccm-conference.2019.199.1.MeetingAbstracts.A7453>

¹⁶ Yasar, O., Long, P., Harder, B., Marshall, H., Bhasin, S., Lee, S., et al. (2022). Machine learning using longitudinal prescription and medical claims for the detection of non-alcoholic steatohepatitis (NASH). BMJ Health & Care Informatics, 29. Retrieved from <https://informatics.bmj.com/content/29/1/e100510>

¹⁷ Doyle, O. M., Leavitt, N., & Rigg, J. A. (2020). Finding undiagnosed patients with hepatitis C infection: An application of artificial intelligence to patient claims data. Scientific Reports, 10, 10521. <https://doi.org/10.1038/s41598-020-67013-6>

¹⁸ Rigg, J., Doyle, O., McDonogh, N., Leavitt, N., Ali, R., Son, A., & Kreter, B. (2023). Finding undiagnosed patients with hepatitis C virus: an application of machine learning to US ambulatory electronic medical records. BMJ health & care informatics, 30(1), e100651. <https://doi.org/10.1136/bmjhci-2022-100651>

¹⁹ IQVIA. (2020). Using artificial intelligence to predict disease progression. Retrieved from <https://www.iqvia.com/library/white-papers/using-artificial-intelligence-to-predict-disease-progression>